

February 23, 2026

Department of Health and Human Services

Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology

Attention: Request for Information: HHS Health Sector AI RFI, Mary E. Switzer Building, Mail

Stop: 7033A

330 C Street SW

Washington, DC 20201

Re: Comment to HHS Health Sector AI RFI RIN 0955-AA13

Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology:

As a Licensed Marriage and Family Therapist for 36 years, I appreciate the opportunity to provide a response to the Department of Health and Human Services (HHS) Request for Information regarding Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care.

Regarding regulation, there is cause for great concern related to the privacy, efficacy, and safety of AI in healthcare. There is no current federal regulation of AI in healthcare. Regulation is essential, given the federal government's obligation to protect the public. This is not something that can be left to the companies producing and selling these products or to individual states, which could result in a patchwork of incompatible regulations. Our government has decided that regulation and oversight are essential in healthcare to ensure safety and efficacy. For example, regulation and appropriate enforcement are important aspects of new products and treatments through the FDA. We regulate healthcare clinicians through licensing requirements. We should not accept anything less with AI and should set the bar higher for unproven technology. AI chatbots are not HIPAA compliant, therefore user/patient privacy is not protected as mandated by federal law in all healthcare industries and settings. Privacy and confidentiality are cornerstones of effective psychotherapy, and the absence of these protections in the use of AI Chatbots demands correction.

Regarding research and development: Due to the novelty of this technology, there is not yet

enough data available to support efficacy and/or safety. Large Language Models (LLMs) are especially prone to "hallucinations," with documented cases involving irrevocable harm of the worst kind. AI LLMs have encouraged vulnerable people struggling with suicide to take suicidal action or provided them with information about the heights of bridges or buildings from which to jump. This is unacceptably risky Chatbot behavior that literally threatens lives. AI products must be thoroughly researched and tested, as is any new psychotherapy protocol or pharmaceutical, to measure effectiveness, dropout rates, remission rates, etc.

Importantly, evaluation, assessment, safety and outcome research should be provided by a separate, independent group of researchers to minimize the conflict of interest inherent if corporations that develop these products also supply the evaluation and “science.” Regarding AI instruments approximating psychotherapy, it’s desirable that the products advance recovery, defined by SAMHSA as a process over time leading to a self-directed life. Is insufficient to focus only on symptom management or reduction. HHS advocacy to accelerate use of AI must follow and be linked to development of an evidence base demonstrating safety and efficacy.

While there are legitimate concerns related to access to mental health care, there are grave risks associated with the use of AI to provide clinical care in the behavioral healthcare industry, especially without rigorous clinical trials, regulation, and robust oversight. At a minimum, HHS should not accelerate the adoption of AI until the FDA completes its evaluations of AI as medical devices. Given the FDA’s findings that well over 10,000 such applications are already widely available to consumers without appropriate premarket performance evaluations, risk management endeavors, or post-market evaluations, HHS must focus on developing research demonstrating safety and efficacy before accelerating implementation.

In study after study over many years, relationship is the main predictor of outcome in psychotherapy. AI is not, in fact, a human relationship, thus it is seriously limited as a potential clinical treatment tool. Much more study and assessment over longer periods of time are necessary. At this point, it is unclear how AI can best be used, whether as a support for psychotherapists, a replacement for psychotherapists, or a tool more tightly focused on insurance processing and administrative issues.

AI Medical devices are not ready for accelerated adoption and deployment without appropriate research or enforceable regulation. Even though LLMs are currently being used by millions of people, this adoption by the public is far ahead of the essential science demonstrating privacy, safety, and efficacy. Hence, regulation is essential to reduce the risk of harm to consumers, ensure that the public has access to safe and effective clinical care, hold corporations accountable for the products they develop and sell, and support uniform national standards for AI Chatbots.

Sincerely,
Barbara Petterson
Licensed Marriage and Family Therapist
Registered Art Therapist